REPORT INFORMATION

Report ID : AMR-20260926-173009

Generated Date : 26-09-2026

Generated Time : 05:30 PM

PATIENT INFORMATION

Patient ID	: PAT-0001	Blood Group	: A-
Patient Name	: sierra	Phone	: 7585428462
Age	: 33	Email	: dallass@karunya.edu.in
Gender	: Female		
Address	: chennai		

CLINICAL INFORMATION

Diagnosis	: Pneumonia
Admission Type	: URGENT
Clinical Notes	: None

LABORATORY INVESTIGATION

Laboratory Test	Result
Creatinine	1.1 mg/dL
Glucose	110.0 mg/dL
Hemoglobin	13.5 g/dL
White Blood Cells	8500.0 /µL
Platelet Count	250000.0 /µL
Potassium	4.2 mmol/L
Sodium	138.0 mmol/L
Urea Nitrogen	18.0 mg/dL

AI PREDICTION RESULT

Prediction	: High AMR Risk	Risk Level	: High
Risk Score	: 54.00 %	Confidence	: 54.00 %

EXPLAINABLE AI (SHAP)

Clinical Feature	SHAP Contribution
organism_count	↑ +0.2246
culture_available	↑ +0.2086
Platelet Count	↑ +0.0399
length_of_stay	↓ -0.0298
admission_type	↓ -0.0212

CLINICAL RECOMMENDATION

Susceptibility Evidence : Susceptible: CEFEPIME, MEROPENEM
Intermediate: CIPROFLOXACIN
Resistant: CEFTAZIDIME, GENTAMICIN

Clinical Actions : Perform Culture & Sensitivity Test
Avoid unnecessary antibiotic use
Review reported antimicrobial susceptibility results
Consider antimicrobial therapy only after clinical and microbiological assessment

Doctor Notes : Susceptibility findings are presented as clinical decision-support evidence. The system does not select or prescribe a specific antibiotic. Final antibiotic selection and treatment decision must

DOCTOR VERIFICATION

Doctor Name	: dallas
Verification Status	: Verified
Verified On	: 26-09-2026 01:47 PM

Notes:
Prediction, SHAP explanation, susceptibility findings and clinical recommendations

Dallas

Scan the QR code to view the clinical report summary.

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DISCLAIMER

This report is generated using an AI-powered Clinical Recommendation Support System.
Recommendations are intended to support healthcare professionals
and should not replace clinical judgment.
Final diagnosis and treatment decisions remain the responsibility
of the treating physician.